

HRS Sensitive Health Data Order Form — Completion Guide

Prepared for **Suhail Bachani** · Merlin Digital (HexaGene project) · 29 June 2026
Form: hrs.isr.umich.edu → Data Products → Downloads → Sensitive Health Data → Sensitive Health Data Order Form

The 3 things this form needs from you:

- User details** — pre-filled from your account (verify the Receiving Entity, see below).
- An attached PDF** — a completed & signed *Sensitive Data Access Use Agreement* (< 2 MB, PDF only). *This is the only blocker.*
- Ticked datasets** — see the checklist on page 2.

1 · User Details — what to enter

Field	Enter	Note
First Name	Suhail	Matches your HRS account.
Last Name	Bachani	—
Email Address	sharad.bachani@merlin-me.com	Your registered login.
Receiving Entity (Institution/Organization)	Merlin Digital (or your exact legal entity name)	You currently have "merln ls" — that looks like a typo. Use the <i>same</i> entity name that appears on your HRS account and on the signed agreement, so all three match.

If name/email are wrong, edit your HRS profile *before* submitting — this form can't request data for other people, and the requester must be the account holder.

2 · The required attachment — Sensitive Data Access Use Agreement

I could not retrieve this template for you — this environment's network policy blocks hrs.isr.umich.edu. But **you can download it directly from the form** (you're already logged in on your own PC).

- On the order form, click **"More information"** next to *Attach PDF* (official template: hrs.isr.umich.edu/sites/default/files/HRS-Sensitive-Data-Access-Use-Agreement.pdf).
- Open it and complete the applicant fields — typically: **name, title/role, institution/organization, address, email, the datasets requested, and your signature + date**. Use the same name/entity as in Section 1.
- It is a self-signed researcher agreement (no separate institutional signing official is required for this Sensitive Health Data tier — unlike the dbGaP genotype track).
- Save the signed file as a **PDF under 2 MB** and attach it in the *Attach PDF* field.

Tip: if your PDF tool produces a file over 2 MB, "print to PDF" again at lower quality or flatten it.

3 · Requested Datasets — tick ALL (your stated goal: maximum coverage)

Every dataset on the form, with a defensible one-line justification tied to the HexaGene validation program. Tier = how directly it ties to a variant-pathogenicity / aging-genomics study (all are reasonable to request; tiers just tell you which are strongest if you're ever asked to trim).

Tick	Dataset	Tier	One-line justification to use
☒	Polygenic Score Data (PGS)	High	Benchmark/triangulate HexaGene-derived variant burden against established genetic risk scores.
☒	APOE & Serotonin Transporter Alleles	High	Directly genotyped loci to anchor variant-scoring and dementia-genetics validation.
☒	HRS Epigenetic Clocks	High	DNA-methylation biological-age phenotypes to test genotype–phenotype associations.
☒	Pace of Aging in HRS (HRSPoA)	High	Rate-of-aging phenotype as an outcome for genetic-score association testing.
☒	2008 Telomere Data	High	Cellular-aging biomarker to correlate with variant-derived measures.

☒	Venous Blood Study (VBS)	High	Rich molecular phenotypes (lipids, inflammation, metabolic) for genotype–phenotype validation.
☒	Biomarker Data	High	Blood-based molecular phenotypes for variant-association validation (already granted — keep ticked for continuity).
☒	Harmonized Cognitive Assessment Protocol (HCAP)	Medium	Deep cognitive phenotyping as an outcome for variant-score association (e.g. AD-related loci).
☒	The Aging, Demographics, and Memory Study (ADAMS)	Medium	Clinically adjudicated dementia outcomes for genotype–phenotype validation.
☒	2003 Diabetes Study	Medium	Metabolic-disease outcome phenotype for variant-score validation.
☒	2019 Health Survey	Medium	Health-status outcomes/covariates for phenotype association models.
☒	Prescription Drug Study (PDS) & Health and Well-Being (HWB)	Low	Medication-use and psychosocial covariates for outcome adjustment.
☒	HRS 2018 Core Module 4: Cannabis	Low	Behavioral/exposure covariate for outcome models.
☒	HRS 2020 Core Module 10: Substance Use Problems	Low	Behavioral covariate for outcome adjustment.

Honest note: requesting all 14 is fine, but the "Low" rows are the ones least obviously tied to a genetics project. If HRS ever queries the request, lead with the Higher genetic/aging datasets; the Low rows can be dropped without affecting the core work.

4 · Reusable research-use justification

Paste a tightened version of this into the agreement's purpose field (and keep it consistent with anything you've told HRS before):

"HexaGene is a deterministic, sequence-intrinsic biophysical score that predicts missense-variant pathogenicity from local DNA thermodynamic and mechanical properties, validated against ClinVar/REVEL using conventional statistics. We will use HRS molecular, biomarker, aging, and health-outcome measures as independent reference phenotypes to validate and calibrate HexaGene-derived variant measures in a deeply phenotyped older-adult cohort. Data will be stored securely, restricted to authorized project personnel, never redistributed, and never used to re-identify or contact participants, in accordance with HRS terms."

5 · AI / LLM Use Policy — you comply (this is in your favor)

HRS's Conditions of Use (updated 02/2026) prohibit using **LLMs and other AI tools** to manage, process, or analyze HRS data. **HexaGene is not affected:** it is a *deterministic, fixed-weight biophysical formula* validated with *classical statistics* (t-tests, Pearson correlation, ROC-AUC). There is no machine-learning model, neural network, or LLM in the scoring or analysis pipeline — exactly the conventional statistical analysis the policy permits.

One tooling caveat (not about your method): do not run the actual downloaded HRS data through any LLM/AI assistant for analysis — that would breach the policy regardless of HexaGene. Keep analysis inside your scipy/HexaGene pipeline. (Using an assistant to fill out *application forms* is fine — that isn't processing HRS data.)

6 · Before you click Submit — checklist

- ☐ Receiving Entity corrected and consistent across form + agreement + account.
- ☐ Signed Sensitive Data Access Use Agreement attached (PDF, < 2 MB).
- ☐ All 14 datasets ticked (or your chosen subset).
- ☐ No passwords or credentials anywhere in the request — request only.
- ☐ Submitted under the account holder's name (Suhail Bachani).

Questions/follow-up: h rsdatareq@umich.edu (requests) · h rsquestions@umich.edu (general). Typical turnaround for Sensitive Health Data: days to ~2 weeks. As a non-US, for-profit requester, expect possible follow-up questions about purpose and data security — the Section 4 statement covers both.